PRACTICE EXAM 24 / EXAM SET 04

Domain 3: Clinical Patient Care

84 scored questions	4 answer choices per question	Answer key + explanations included

Instructions

Choose the single best answer (A, B, C, or D) for each question. Mark your responses before checking the answer key and detailed explanations at the end of this document. These practice questions are not official NHA examination items.

QUESTIONS

Q01 | Patient Intake and Vitals

> **SCENARIO** A blood pressure cuff has an artery marker that its instructions require aligning with the brachial artery. The marker is currently facing the back of the upper arm rather than the specified anterior-medial position.

What should the assistant correct?

A. Leave the marker at the back and increase cuff inflation pressure.
B. Place the stethoscope over the marker at the back regardless of artery location.
C. Rotate and reapply the cuff so the marker aligns with the specified artery position.
D. Use the cuff upside down without checking the remaining placement instructions.

Q02 | General Patient Care

> **SCENARIO** A patient with an indwelling urinary catheter is being assisted into a wheelchair. The assistant is trained and authorized for the routine bag-positioning task. Clinic procedure keeps the closed drainage system intact, the bag below the bladder, and the bag off the floor. The bag is currently on the patient's lap.

Which preparation follows the procedure?

A. Disconnect the tubing temporarily so the bag is easier to carry.
B. Leave the bag on the lap above bladder level during the trip.
C. Set the bag on the floor so it is well below the bladder.
D. Secure the bag in its appropriate holder below bladder level without disconnecting the system.

Q03 | Infection Control and Safety

> **SCENARIO** A wall-mounted liquid-soap dispenser is partly empty. A coworker proposes pouring fresh soap into the remaining soap to fill it. The dispenser has a manufacturer-approved replacement and cleaning procedure.

Which action is appropriate?

A. Add fresh soap directly to the remaining soap without processing the dispenser.
B. Mix different soaps in the dispenser to strengthen their effect.
C. Add water to the remaining soap to restore the full level.
D. Follow the approved dispenser procedure rather than topping off the partly filled soap.

Q04 | General Patient Care

> **SCENARIO** A sterile-field procedure defines the outer 1-inch border of the wrapper as nonsterile. A sterile forceps has been placed entirely on that border rather than in the sterile central area.

How should the forceps be handled?

A. Treat it as contaminated and obtain a sterile replacement.
B. Move it from the border to the center and consider it sterile again.
C. Ask the patient whether it appears clean before deciding.
D. Use it if only the handle lies within the border.

Q05 | Infection Control and Safety

> **SCENARIO** A reusable instrument must be sterile because it enters sterile tissue. Its manufacturer prohibits steam and validates a specific low-temperature sterilization method. The clinic has only a steam unit but can arrange the validated method through its approved processing service.

What should the assistant arrange?

A. Substitute high-level disinfection for the required validated sterilization.
B. Use the clinic steam cycle despite the manufacturer's prohibition.
C. Choose an unvalidated cooler steam cycle without obtaining an approved process.
D. The validated compatible sterilization process through the approved service before use.

Q06 | Phlebotomy

SCENARIO A qualified clinician is administering an IV infusion in a patient's left arm. A venous specimen is ordered, and clinic collection procedure directs a suitable opposite-arm site when available. The assistant is not authorized to stop or adjust the infusion. A suitable right-arm site is available.

Which approach follows this procedure?

A. Collect from the infusion tubing without the required authorization.
B. Collect from the suitable right arm without independently changing the infusion.
C. Stop the infusion independently and collect above its insertion site.
D. Collect from the left infusion site and relabel the specimen as right-arm blood.

Q07 | Point of Care Testing and Laboratory Procedures

SCENARIO A specified CLIA-waived test uses a new control lot with an acceptance range of 100-120 units. An old lot's sheet lists 80-100 units. Today's new-lot control result is 90 units. Instructions require the current lot's range and acceptable QC before patient testing.

Which evaluation is appropriate?

A. Release patient results before verifying the new-lot control range.
B. Accept 90 using the obsolete 80-100 range sheet.
C. QC fails against the current 100-120 range; hold testing and follow corrective instructions.
D. Use whichever lot's range produces a passing value today.

Q08 | Infection Control and Safety

SCENARIO A staff lunch container is placed in a refrigerator used to hold patient blood specimens. A separate food refrigerator is available outside the specimen area.

Which correction is appropriate?

A. Remove the food from specimen storage and use the designated food-storage area.
B. Keep the lunch there if its lid is closed.
C. Move the blood tubes into the lunch container to separate them from the food.
D. Keep food on the upper shelf and specimens below it as the sole correction.

Q09 | Patient Intake and Vitals

SCENARIO A stable patient's ordered orthostatic measurements are 138/82 mm Hg supine and 116/76 mm Hg standing. The supplied reporting rule flags a systolic fall of at least 20 mm Hg or a diastolic fall of at least 10 mm Hg.

Which finding meets the stated reporting rule?

A. Neither pressure change, because the rule requires symptoms.

B. Only the 6 mm Hg diastolic fall meets the rule.

C. Neither pressure change, because both threshold limits must be met together.

D. The 22 mm Hg systolic fall.

Q10 | General Patient Care

SCENARIO Immediately before an authorized medication task, the EHR displays a newly entered allergy alert for the ordered drug. The order predates the alert, and no clinician has reviewed the new information. The patient has not received the dose.

What should the assistant do?

A. Pause administration and obtain clinician review of the new alert before proceeding.

B. Delete the allergy information so the existing order can proceed.

C. Administer a smaller amount without prescriber clarification.

D. Override the alert because the earlier prescription was signed.

Q11 | General Patient Care

SCENARIO A responsive adult man suddenly cannot speak or cough effectively after eating. He is standing, the trained assistant can encircle his abdomen, and another staff member has activated emergency response. The assistant follows current 2025 AHA-based clinic training.

Which intervention sequence is indicated while he remains responsive with severe obstruction?

A. Encourage cough alone despite the described inability to cough effectively.

B. Use abdominal thrusts alone without the back-blow cycles in current training.

C. Use blind finger sweeps before the trained obstruction-response cycles.

D. Alternate five back blows with five abdominal thrusts as trained.

Q12 | Point of Care Testing and Laboratory Procedures

> **SCENARIO** A specified waived assay requires room operation at 15-30 degrees C. A clinic room measures 35 degrees C before testing, and an appropriate temperature-controlled testing area is available.

What should the assistant do?

A. Establish the required operating conditions before testing under the assay instructions.

B. Use the out-of-range room because the supplies are unopened.

C. Ignore room temperature if the patient specimen looks normal.

D. Run the assay and adjust results downward to compensate for heat.

Q13 | Point of Care Testing and Laboratory Procedures

> **SCENARIO** An ordered pre- and post-medication spirometry protocol requires a baseline before the authorized inhaled medication and a second set 15 minutes afterward. The baseline is complete, and the medication is given at 11:20 a.m. The assistant is trained and authorized for the ordered tasks.

Which planned second-set time follows the protocol?

A. 11:25 a.m.

B. 11:20 a.m.

C. 11:35 a.m.

D. 12:20 p.m.

Q14 | Patient Intake and Vitals

> **SCENARIO** A comfortable patient cannot keep the mouth closed because of persistent mouth breathing. The oral thermometer instructions require a closed mouth. Clinic protocol permits an available temporal device when oral technique cannot be completed.

Which approach follows these instructions?

A. Use the permitted temporal device and document the actual measurement route.

B. Take an oral reading with the mouth open and label it as standard oral technique.

C. Use the oral value and add a guessed correction for mouth breathing.

D. Ask the patient to stop breathing long enough to obtain a reading.

Q15 | Point of Care Testing and Laboratory Procedures

> **SCENARIO** During a pure-tone hearing screen, a patient waits until each tone ends before raising a hand. The approved response instruction is to signal as soon as a tone is heard, even if it is faint.

Which coaching best matches the procedure?

A. Raise your hand every few seconds whether or not a tone is heard.
B. Signal when you first hear the tone, even if it is faint.
C. Watch the assistant's face and signal whenever the assistant looks up.
D. Wait until each tone has stopped before responding.

Q16 | EKG and Cardiovascular Testing

> **SCENARIO** An irregular EKG contains 9 QRS complexes in a 6-second recording window. The assistant is asked to estimate the mean ventricular rate from the full window rather than from one variable R-R interval.

What is the estimated mean rate?

A. 900 beats/min
B. 9 beats/min
C. 54 beats/min
D. 90 beats/min

Q17 | General Patient Care

> **SCENARIO** A trained, authorized assistant is removing skin staples under an order. The supplied remover instruction says to place the lower jaw beneath the staple center and squeeze the handles so the center bends and the ends lift. The lower jaw is positioned correctly.

Which action follows this device instruction?

A. Squeeze as instructed to bend and lift the staple.
B. Pull straight upward without operating the handles.
C. Cut the skin around the staple with scissors.
D. Rotate the closed staple sideways through the skin.

Q18 | Phlebotomy

SCENARIO During venipuncture, a fragile vein visibly collapses when a high-vacuum tube is applied, and the same problem occurs with a second tube. The clinic plan calls for stopping safely and selecting its approved lower-vacuum, small-volume method for the next attempt rather than probing or increasing suction.

Which plan addresses the observed problem?

A. Keep the same method and probe deeper while another high-vacuum tube is applied.
B. Stop safely and use the approved lower-vacuum, small-volume approach under the collection plan.
C. Repeat the same high-vacuum approach without the required alternate-method step.
D. Apply stronger suction to overcome the observed collapse.

Q19 | Patient Intake and Vitals

What is the usual adult apical pulse listening location when using a stethoscope?

A. Right second intercostal space at the sternal border.
B. Midline just above the suprasternal notch.
C. Left second intercostal space at the sternal border.
D. Left fifth intercostal space at the midclavicular line.

Q20 | General Patient Care

SCENARIO Before a video visit, the clinician needs to see the patient's face clearly. A bright window behind the patient makes the face appear as a dark silhouette, although the connection works.

Which setup change best addresses the problem?

A. Keep the silhouette and document a completed visible-face assessment.
B. Ask the patient to turn the camera away from the face.
C. Reposition the patient or lighting so light falls onto the face rather than from behind it.
D. Increase audio volume instead of correcting the image.

Q21 | General Patient Care

SCENARIO After a contusion, the provider's approved plan specifies a cold pack with a cloth barrier for 10 minutes at the stated interval. The patient plans to place the pack directly on skin for an hour.

Which reinforcement follows the plan?

A. Use the cloth barrier and the prescribed 10-minute application period.
B. Extend the application to an hour because longer cold is always better.
C. Leave the pack directly on the skin until the area is numb.
D. Replace the cold pack with a heating pad without checking the plan.

Q22 | General Patient Care

> **SCENARIO** A provider requests a pair of sterile scissors during a minor procedure. The clinic passing technique specifies closed blades directed away from the receiver and presentation of the finger rings to the receiver.

Which transfer follows this technique?

A. Hold the scissors by the rings and push the open tips into the receiving hand.

B. Pass the closed scissors with the finger rings presented to the provider.

C. Present the open blade tips toward the provider's hand.

D. Set the open scissors against the patient while the provider reaches for them.

Q23 | Infection Control and Safety

What processing is appropriate for a reusable blood pressure cuff contacting only intact skin, using compatible manufacturer instructions?

A. Cleaning followed by the compatible low-level disinfection process.

B. Treating the cuff as an implant and requiring an incompatible sterilization cycle.

C. Immersion in an unapproved high-level chemical without checking compatibility.

D. Detergent cleaning alone with no required disinfection.

Q24 | General Patient Care

> **SCENARIO** Two separately authorized vaccines are administered: product A in the left deltoid and product B in the right deltoid. The assistant is completing the administration records.

Which site documentation preserves the product-to-site relationship?

A. Assign the left deltoid to both products because both are intramuscular.

B. Omit sites because the lot numbers identify where the doses were given.

C. Record only "both arms" as a shared site for both products.

D. Record product A with the left deltoid and product B with the right deltoid.

Q25 | Patient Intake and Vitals

> **SCENARIO** A patient says, "I have not felt feverish." The assistant then measures a temperature of 38.9 degrees C using correct technique. The provider will evaluate the patient.

How should the assistant document these observations?

A. Change the patient's words to "reports fever" so both entries agree.

B. Record the patient's report and the measured temperature separately, and report the finding as required.

C. Diagnose the cause of the temperature from this information alone.

D. Delete the temperature because it conflicts with the patient's report.

Q26 | Point of Care Testing and Laboratory Procedures

> **SCENARIO** An ordered semen specimen kit instructs collection directly into the supplied container and prohibits ordinary condoms or unapproved lubricant. Before collection, the patient asks to use a standard lubricated condom and transfer the contents later.

Which instruction follows the kit?

A. Use the ordinary condom because transfer makes the original container irrelevant.
B. Mix the sample with tap water before transfer.
C. Use the supplied collection container and follow the kit's preparation directions.
D. Add lubricant to the supplied container to increase the volume.

Q27 | Patient Intake and Vitals

> **SCENARIO** During medication history, a patient says a home injection pen is set to "ten clicks." The pen and its instructions are unavailable, and the assistant cannot verify what volume or dose those clicks represent.

How should the assistant handle this report?

A. Remove the medication from the history because the dose is uncertain.
B. Convert ten clicks to ten units without identifying the device.
C. Document the patient's exact description and flag the unresolved dose for clinician verification.
D. Record ten milliliters because clicks must correspond to volume.

Q28 | Phlebotomy

> **SCENARIO** The laboratory requires an aliquot to retain the original specimen accession and collection time, with the aliquot-preparation time in a separate field. A specimen was collected at 9:00 a.m. and aliquoted at 9:45 a.m.

Which labeling record follows the requirement?

A. Replace the collection time with 9:45 a.m. and discard the original accession.
B. Retain the original accession and 9:00 a.m. collection time, and record 9:45 a.m. separately as aliquot preparation.
C. Assign an unrelated accession to make the aliquot look like a new collection.
D. Use only the assistant's initials because the parent tube is nearby.

Q29 | Infection Control and Safety

SCENARIO A device's high-level disinfection method requires its internal channel to be completely filled with solution for the full contact time. An air pocket remains in the channel although the outside is submerged. The assistant is trained in the approved method.

What should the assistant do before starting the required exposure timer?

A. Eliminate the air pocket and fill the channel according to instructions, then complete the full exposure.

B. Count full contact from outer immersion even though the channel was not filled.

C. Start exposure timing while the channel still contains the visible air pocket.

D. Leave the air pocket and estimate an unvalidated time extension.

Q30 | Infection Control and Safety

SCENARIO An injection station is ready except that there is no approved sharps container within the designated point-of-use location. A portable approved container is available. No needle has yet been used.

What should the assistant do before starting?

A. Plan to carry the used needle uncovered across the room.

B. Place and secure the approved sharps container at the required point-of-use location.

C. Leave the used needle on a tray until the station is closed.

D. Use a drinking cup temporarily and sort the sharps later.

Q31 | General Patient Care

SCENARIO An authorized, trained assistant is preparing a clinically verified non-IV dose of a nonvaccine medication: 600 mg. Each new single-dose vial contains 1.5 mL at 300 mg/mL. The task is to determine the minimum unopened-vial supply needed for this one patient; unused medication will be handled under policy.

What is the minimum number of vials required?

A. 2 vials

B. 1 vial

C. 4 vials

D. 3 vials

Q32 | General Patient Care

SCENARIO A provider-approved instruction for an ordered metered-dose inhaler with spacer says to release one puff into the spacer and inhale it as directed before the next puff. The patient plans to release all prescribed puffs into the spacer together.

Which reinforcement matches the supplied technique?

A. Double the number of puffs to compensate for releasing them together.
B. Release every puff together and leave them in the chamber until later.
C. Remove the spacer and spray toward the cheek instead of using the ordered method.
D. Release and inhale one puff at a time, following the prescribed interval and technique.

Q33 | EKG and Cardiovascular Testing

SCENARIO At ambulatory-monitor return, staff verify that the recorder clock ran one hour behind the correct clinic time. The patient's diary used the correct time. Raw recordings and diary entries are available for review.

Which handling supports accurate event correlation?

A. Silently rewrite the patient's diary to hide the discrepancy.
B. Preserve the originals and flag the verified clock offset for the reviewer.
C. Assume every diary symptom is false because the clocks differ.
D. Delete the recordings made during diary-reported events.

Q34 | General Patient Care

SCENARIO A provider orders compression stockings after confirming suitability. The fitting guide requires both ankle and calf measurements to fall within one size. Medium accepts ankle 22-26 cm and calf 34-40 cm; large accepts ankle 26-30 cm and calf 40-46 cm. The patient measures 24 cm at the ankle and 44 cm at the calf.

Which fitting decision follows the guide?

A. Choose medium and roll the upper band tightly to compensate.
B. Obtain approved fitting guidance because neither available size matches both measurements.
C. Select medium based only on the ankle measurement.
D. Select large based only on the calf measurement.

Q35 | Phlebotomy

SCENARIO For a specific capillary microcollection, the laboratory guide lists EDTA before serum and warns staff not to substitute the venous tube sequence. A coworker plans to collect serum first by habit.

Which action follows this collection-specific instruction?

A. Label both specimens as venous to justify the planned sequence.
B. Pour part of the serum specimen into the EDTA container afterward.
C. Use the venous sequence regardless of specimen method.
D. Use the stated capillary sequence, with EDTA before serum.

Q36 | Patient Intake and Vitals

SCENARIO A child can use the clinic's approved faces pain scale but asks whether to choose the face that looks happiest. The scale instructions say to select the face representing how much the child hurts, not mood.

Which explanation best supports valid self-report?

A. Let the assistant choose based on how much you are crying.
B. Choose the face that shows how much you hurt, even if your mood is different.
C. Choose the middle face so the result is easier to compare.
D. Choose the happiest face whenever you feel cheerful.

Q37 | Phlebotomy

SCENARIO The clinic limits an assistant to two unsuccessful venipuncture attempts on one patient before contacting the designated lead for further collection arrangements. Two attempts have failed, and the patient remains stable.

What should the assistant do next?

A. Make a third attempt with a fresh needle without contacting the designated lead.
B. Ask an unapproved helper to continue attempts outside the clinic process.
C. Stop further attempts and contact the designated lead under the policy.
D. Continue probing through the second puncture instead of treating it as unsuccessful.

Q38 | General Patient Care

SCENARIO A calm child will receive an ordered injection while seated on a caregiver's lap. The clinic's trained comfort-hold method supports the child and keeps the injection limb steady without restricting breathing. The assistant and caregiver have been instructed in it.

Which preparation follows this method?

A. Allow the injection limb to swing freely while the needle is inserted.
B. Ask an untrained bystander to force the child flat without using the approved method.
C. Use the trained supportive hold and pause for help if a safe steady position cannot be maintained.
D. Press the child's face into the caregiver's chest to prevent movement.

Q39 | Point of Care Testing and Laboratory Procedures

> **SCENARIO** A test's supplied waived-method instructions accept capillary whole blood only. The clinic receives a tube of venous plasma, and no authorized alternative method is available in the clinic.

What should the assistant do?

A. Run the plasma under the capillary method because both originated from blood.

B. Obtain instructions for an acceptable specimen or appropriately authorized laboratory route without modifying the waived method.

C. Relabel the plasma as capillary whole blood.

D. Mix the plasma with water to approximate the specified specimen.

Q40 | General Patient Care

> **SCENARIO** Before a provider-performed procedure involving sedation, the clinic checklist requires confirmation of a responsible adult escort before sedating medication is given. The patient says no escort is available and plans to drive home. The assistant cannot waive this requirement.

What should the assistant do?

A. Treat the patient's access to a car as an escort substitute.

B. Flag the unmet requirement and obtain the procedural clinician's direction before sedation proceeds.

C. Start the planned sequence and ask about an escort only after sedation.

D. Independently promise discharge without an escort if the patient later feels alert.

Q41 | Phlebotomy

> **SCENARIO** A drug-level collection order specifies a specimen exactly 2 hours after an observed oral dose. The documented dose time is 8:40 a.m. No other timing condition applies.

At what time should the specimen be scheduled?

A. 10:40 a.m.

B. 8:40 a.m.

C. 9:40 a.m.

D. 12:40 p.m.

Q42 | General Patient Care

> **SCENARIO** A trained, authorized assistant has a clinically verified order for 125 mg of an oral liquid. Its verified label states 5% weight/volume, defined on the label as 5 g per 100 mL. Use 1 g = 1,000 mg.

What volume provides the ordered amount?

A. 25 mL
B. 0.25 mL
C. 2.5 mL
D. 0.025 mL

Q43 | Infection Control and Safety

> **SCENARIO** A disinfecting chemical's safety data sheet requires a specified chemical-resistant glove for the task and states that ordinary thin examination gloves are unsuitable. The required gloves are available.

Which glove choice follows the safety data sheet?

A. Two pairs of the unsuitable examination gloves without approval.
B. The specified chemical-resistant gloves.
C. Ordinary examination gloves because all medical gloves resist all chemicals.
D. Bare hands for a shorter exposure time.

Q44 | General Patient Care

> **SCENARIO** An assistant removes the protective cap from a new medication vial before an authorized injection. The clinic preparation instruction requires cleaning the rubber stopper before needle entry, including on newly opened vials.

What should the assistant do before entry?

A. Skip preparation because the protective cap guarantees a sterile stopper.
B. Clean the stopper using the instructed aseptic preparation.
C. Touch the stopper with a fingertip to confirm it is dry and clean.
D. Remove the entire stopper and pour the medication into the syringe.

Q45 | Infection Control and Safety

> **SCENARIO** A completed packaged sterilizer load is still hot. The validated handling instructions require full cooling in the designated clean area before handling and storage.

Which action follows the instructions?

A. Stack the hot packages tightly on a cold wet counter to cool them faster.
B. Allow the packages to cool fully in the designated area before handling them for storage.
C. Handle the hot packs with damp towels and return them to stock.
D. Open each hot package so the instruments cool directly in the room air.

Q46 | General Patient Care

Which instrument provides a vibrating stimulus during a sensory examination?

A. The tuning fork.
B. The reflex hammer.
C. The tongue depressor.
D. The otoscope.

Q47 | General Patient Care

> **SCENARIO** An assistant performed ordered wound care at 10:00 a.m. An urgent interruption delayed documentation until 1:15 p.m. The EHR retains the current entry timestamp and allows the actual service time to be recorded.

Which documentation is appropriate?

A. Leave the care undocumented because the original time has passed.
B. Change the system timestamp so it appears the note was entered at 10:00 a.m.
C. Enter the care now, retain the entry timestamp, and identify 10:00 a.m. as the actual service time.
D. Record the wound care as occurring at 1:15 p.m. even though that was only the entry time.

Q48 | Phlebotomy

> **SCENARIO** A capillary hemoglobin collection device has a visible air bubble in its optical filling area. Its instructions require a complete continuous fill without bubbles and prohibit topping up a partially filled device.

What should the assistant do?

A. Ignore the bubble because the patient's name is correct.
B. Add a second drop to the partially filled device despite the prohibition.
C. Tap the device aggressively until the sample splashes from the filling area.
D. Use a new device and obtain a proper continuous fill under the instructions.

Q49 | EKG and Cardiovascular Testing

> **SCENARIO** During EKG acquisition, the displayed tracing becomes flat. The assistant immediately checks the patient, who is awake, speaking, and has a palpable pulse. The clinic recording procedure calls for checking connections and escalating unresolved recording faults in this situation.

Which action follows that assessment?

A. Begin chest compressions from the display alone despite the confirmed responsive patient.
B. Record cardiac arrest as established while disregarding the patient assessment.
C. Check the recording connections and follow the unresolved-fault process if needed.
D. Delay checking the connections and assign a rhythm diagnosis from the flat display alone.

Q50 | General Patient Care

> **SCENARIO** After wound evaluation, a provider orders removal of alternate sutures only, leaving the others until reassessment. The sutures are clearly identified, and the trained, authorized assistant sees no new wound problem.

Which implementation follows the order?

A. Change the planned reassessment interval independently after removal.
B. Remove only the specified alternate sutures and leave the remainder for the ordered reassessment.
C. Remove only the knots while leaving unidentified fragments in the tissue.
D. Remove every suture because the wound appears closed.

Q51 | Point of Care Testing and Laboratory Procedures

> **SCENARIO** A laboratory calls with an authenticated amendment: a previously released potassium result of 5.9 mmol/L is corrected to 4.2 mmol/L. The ordering clinician already received the earlier report. Clinic procedure requires promptly relaying authenticated amendments and documenting the correction and notification.

What should the assistant do?

A. Notify the patient of a treatment change chosen by the assistant.
B. Preserve only the first value and ignore the authenticated correction.
C. Relay the authenticated corrected result promptly and document it through the approved amendment workflow.
D. Enter the amendment silently without the required active notification.

Q52 | EKG and Cardiovascular Testing

How many surface electrodes are used for a standard resting 12-lead EKG?

A. 12 electrodes
B. 6 electrodes
C. 4 electrodes
D. 10 electrodes

Q53 | General Patient Care

> **SCENARIO** A provider orders two different vaccine products as separate injections. They are not a licensed combination, and the instructions do not authorize mixing. The assistant is trained and authorized for the ordered injections.

Which preparation follows the orders?

A. Prepare each product separately in its own labeled syringe.
B. Administer only the combined volume of whichever product is easier to draw.
C. Mix the products in one syringe to reduce the number of punctures.
D. Transfer one prepared vaccine into the other syringe without checking compatibility.

Q54 | Phlebotomy

SCENARIO A blood ethanol collection kit specifies the supplied nonalcohol skin preparation for this assay. The assistant also has routine alcohol pads. All other kit requirements are met.

Which preparation should be selected?

A. A routine alcohol pad instead of the specified kit preparation.

B. The supplied nonalcohol skin preparation specified for the ethanol specimen.

C. Any scented cleanser because only the tube additive matters.

D. An alcohol pad followed by a label saying nonalcohol preparation.

Q55 | General Patient Care

SCENARIO Under applicable law and clinic policy, the trained assistant is authorized to administer an ordered non-IV controlled medication. The unused remainder requires witnessed disposal before the waste record is signed. The required witness is temporarily unavailable, and the policy specifies secure holding until the witness arrives.

What should the assistant do?

A. Leave the remainder unattended while searching for a witness.

B. Secure the remainder as directed and complete disposal with the required witness before signing the waste record.

C. Keep the remainder secure but sign the completed waste record before witnessed disposal.

D. Discard alone and request a later witness signature from someone who did not observe.

Q56 | Infection Control and Safety

SCENARIO A biological-monitor system requires an unprocessed same-lot positive control to show growth after incubation. The processed test shows no growth, but the unprocessed positive control also shows no growth. The instructions classify this combination as an invalid monitoring run.

Which interpretation is appropriate?

A. Treat the failed positive control as evidence that incubation is unnecessary.

B. Report successful sterilization based only on the processed no-growth container.

C. Report an invalid monitoring run and follow the required review process.

D. Ignore the non-growing positive control because it stayed outside the load.

Q57 | Infection Control and Safety

SCENARIO An enzymatic instrument cleaner's directions require preparation at 20-30 degrees C and warn against hotter water. The assistant prepares a fresh bath measuring 55 degrees C.

Which correction follows the directions?

A. Add extra concentrate to compensate for excessive temperature.
B. Shorten the cleaning step while retaining the out-of-range bath.
C. Use the hot bath because heat always improves an enzymatic cleaner.
D. Prepare the solution within the specified temperature range before use.

Q58 | Point of Care Testing and Laboratory Procedures

SCENARIO At the standard 20 ft distance, a patient reads the chart line labeled 40 ft. The clinic records the test distance as numerator and the line designation as denominator.

Which visual-acuity entry follows this convention?

A. 40/40
B. 20/40
C. 40/20
D. 20/20

Q59 | General Patient Care

SCENARIO During an authorized oral medication task, a patient coughs repeatedly after a small sip and says swallowing now feels difficult. The patient remains responsive and breathing, but the next medication has not been given. The clinic procedure requires stopping oral tasks and obtaining clinical review for new swallowing difficulty.

What should the assistant do?

A. Ignore the new difficulty because the oral route was ordered earlier.
B. Stop further oral administration and promptly obtain the required clinician review.
C. Give the remaining tablets quickly while the patient is still alert.
D. Crush the remaining medication into food without verifying the formulation or order.

Q60 | EKG and Cardiovascular Testing

SCENARIO An assistant suspects V1 and V2 were placed too high because the intercostal count began at the clavicle. The clinic standard technique identifies the sternal angle and second rib before counting to the required spaces.

Which correction best supports accurate re-placement?

A. Move both electrodes down an arbitrary distance without identifying landmarks.
B. Leave them in place because an electrode that adheres must be correctly positioned.
C. Use the nipple height as the sole reference for both electrodes.
D. Restart the anatomical count at the sternal angle and second rib as directed.

Q61 | Patient Intake and Vitals

SCENARIO A newborn weighs 6 lb 10 oz. The intake system requires kilograms to two decimal places. Use 16 oz = 1 lb and 2.2 lb = 1 kg; round only the final answer.

Which weight should be entered?

A. 3.31 kg
B. 3.01 kg
C. 14.58 kg
D. 2.73 kg

Q62 | Patient Intake and Vitals

SCENARIO A toddler needs pulse oximetry. The adult sensor does not fit securely, and the device instructions list a compatible pediatric sensor for the child's age and weight. That sensor is available.

Which choice best supports the planned measurement?

A. Hold the oversized adult sensor loosely against the finger and accept any number.
B. Use the listed compatible pediatric sensor and verify an adequate signal.
C. Place the adult sensor over a thick bandage to improve its fit.
D. Record the adult sensor's default display without checking contact.

Q63 | EKG and Cardiovascular Testing

SCENARIO Before patient acquisition, the EKG manufacturer's check applies a known 1 mV test signal at 10 mm/mV. Its acceptance band is a 9-11 mm printed deflection, but the check produces 8 mm. The instructions require service review after a failed check.

What should the assistant do?

A. Apply an unapproved numerical correction to later patient recordings.
B. Change the printed calibration label without resolving the failed check.
C. Keep the device out of patient acquisition and arrange the required equipment review.
D. Use the device because the displayed gain setting is correct despite the failed output.

Q64 | Infection Control and Safety

SCENARIO A cleaning chemical is transferred to a reusable secondary bottle for later staff use. The clinic hazard-communication policy requires the product identity and specified hazard information on that bottle; no immediate-use exception applies.

Which labeling action follows the policy?

A. Apply the required product identity and hazard information before the bottle enters use or storage.
B. Use a blank bottle because the chemical is recognizable by color.
C. Label it with a different familiar product name to simplify ordering.
D. Label it only with the employee's first name.

Q65 | Infection Control and Safety

SCENARIO Before a task involving splash-risk chemicals, the assistant notices supply boxes blocking direct access to the eyewash station. Clinic safety procedure requires unobstructed access while these chemicals are handled.

What should happen before the task begins?

A. Clear the eyewash access path under the safety procedure.
B. Place more boxes in front to prevent accidental eyewash activation.
C. Leave the boxes because an exposure is unlikely.
D. Plan to search for another sink only if a splash occurs.

Q66 | General Patient Care

SCENARIO A prescribed oral suspension has separated into layers while standing. The product instructions require shaking to redistribute the medication before each dose, and there is no other identified product problem. The assistant is trained and authorized for administration.

What preparation is appropriate?

A. Measure only the dense lower layer to obtain the strongest dose.
B. Add water to the bottle to remove the separation without checking instructions.
C. Measure only from the clear upper layer to avoid sediment.
D. Shake the suspension as instructed before measuring the verified dose.

Q67 | Phlebotomy

SCENARIO A capillary tube is being prepared for an authorized microhematocrit workflow. The device instructions require sealing the designated end with the supplied sealant and checking the seal before centrifugation. The end is still open.

Which preparation is needed?

A. Spin the open tube because the rotor will keep the blood inside.
B. Cover the open end loosely with a patient label.
C. Seal the designated end with the supplied material and verify it before centrifugation.
D. Insert a needle into the open end and leave it there during spinning.

Q68 | General Patient Care

SCENARIO A provider requests Sims positioning for a patient's examination and specifies lying on the left side with the upper knee flexed. The patient can move safely.

Which setup follows that order?

A. Place the patient supine with both legs in stirrups.
B. Place the patient prone with both legs straight.
C. Seat the patient upright with feet flat on the floor.
D. Help the patient into the specified left-side position with the upper knee flexed.

Q69 | Phlebotomy

SCENARIO After an infant heel collection, clinic aftercare procedure requires clean gauze pressure until bleeding stops and avoids a loose adhesive bandage because it could become a choking item. The puncture still needs brief pressure.

Which aftercare follows this procedure?

A. Maintain the instructed clean-gauze pressure and confirm bleeding stops without adding a loose adhesive bandage.
B. Offer the infant the gauze to chew while the caregiver prepares to leave.
C. Stop pressure immediately because a bandage is not being used.
D. Apply a loose bandage and leave it for the infant to remove.

Q70 | Phlebotomy

> **SCENARIO** An order requires two blood-culture sets obtained from two separate venipuncture sites, with each site identified in the collection record. The trained assistant proposes filling both sets through one puncture and labeling them as two sites.

Which plan follows the order?

A. Fill both through one puncture and invent a second site label.
B. Omit site information because the two sets share a patient name.
C. Obtain the sets from the two separately prepared ordered sites and document each actual site.
D. Pool the blood from one site and divide it between labels afterward.

Q71 | General Patient Care

> **SCENARIO** A provider orders cleansing of a closed incision from the incision line outward, using a fresh gauze surface for each pass and not returning a used surface to the incision. The assistant is trained and authorized for this wound-care task.

Which technique follows the order?

A. Move a used gauze surface from surrounding skin back across the incision.
B. Use one visibly soiled surface for all passes to conserve supplies.
C. Clean outward from the incision with fresh gauze surfaces for successive passes.
D. Scrub surrounding drainage into the incision before moving outward.

Q72 | Infection Control and Safety

> **SCENARIO** A discarded specimen item contains both biological material and a chemical preservative. The clinic mixed-hazard procedure requires the designated combined-hazard route and prohibits placing it in an ordinary blood-waste bag or pouring it into a sink.

Which disposal route follows the procedure?

A. The sink, followed by an empty-container rinse.
B. A clean paper recycling bin after the label is removed.
C. The designated combined biological-and-chemical hazard route.
D. A routine blood-waste bag without considering the preservative.

Q73 | Patient Intake and Vitals

> **SCENARIO** A 78-year-old wheelchair-dependent patient cannot stand safely for weighing. The clinic has an accessible wheelchair scale and trained staff for its documented chair-tare procedure.

Which method is appropriate?

A. Use the accessible scale with the specified chair-tare procedure.
B. Use the caregiver's estimate as a directly measured weight.
C. Lift the patient onto a standard standing scale without the planned transfer support.
D. Ask the patient to stand briefly without support to avoid using special equipment.

Q74 | Point of Care Testing and Laboratory Procedures

> **SCENARIO** A clinic microscope has residue on an optical lens. The device cleaning instruction specifies approved lens solution and lens paper and prohibits abrasive towels. The assistant is performing authorized equipment care.

Which cleaning choice follows the instruction?

A. Apply any oily lubricant to hide the residue.
B. Use the approved lens solution and lens paper.
C. Scrape the lens with a metal instrument.
D. Use a rough paper towel with surface bleach.

Q75 | General Patient Care

> **SCENARIO** An assistant is trained and authorized to administer prescribed eye drops. The approved technique directs the drop into the lower conjunctival sac without touching the eye with the applicator.

Where should the drop be directed?

A. Into the lower conjunctival sac as instructed.
B. Directly onto the central cornea with the applicator touching the eye.
C. Onto the outer skin at the eye corner and wiped away immediately.
D. Onto the closed eyelid without exposing the conjunctival sac.

Q76 | Patient Intake and Vitals

> **SCENARIO** During infant rooming, the assistant observes a prolonged breathing pause with dusky color. The clinic emergency procedure requires immediate response for a breathing pause accompanied by color change, regardless of a routine appointment reason.

What should the assistant do?

A. Record the pause as normal without notifying anyone.
B. Finish measuring length before reporting the observation.
C. Wait for the next routine respiratory count because infants sometimes breathe irregularly.
D. Activate the clinic emergency response and summon immediate clinical help.

Q77 | Patient Intake and Vitals

SCENARIO During an adult manual blood pressure measurement, the first clear Korotkoff sounds occur at 158 mm Hg. They briefly disappear from 140 to 130 mm Hg, return, and finally disappear at 82 mm Hg. The supplied procedure uses first sound for systolic pressure and final disappearance for diastolic pressure.

Which reading follows this procedure?

A. 158/130 mm Hg
B. 130/82 mm Hg
C. 158/82 mm Hg
D. 140/82 mm Hg

Q78 | General Patient Care

SCENARIO A patient with a confirmed low glucose reading becomes drowsy and cannot swallow safely. A prior standing order for oral glucose applies only when the patient is alert and able to swallow. The clinician and emergency team are available.

What should the assistant do?

A. Withhold oral intake and summon immediate clinical help under the emergency procedure.
B. Apply the oral-glucose order despite the newly unsafe swallowing condition.
C. Place glucose inside the cheek and delay clinical notification until it dissolves.
D. Wait for a later routine glucose check before seeking help for the new condition.

Q79 | Infection Control and Safety

SCENARIO A reusable surgical instrument completes an ultrasonic cleaning cycle. It will enter sterile tissue, and its approved reprocessing sequence includes inspection, packaging, and validated sterilization after cleaning. A coworker wants to place it directly in sterile stock.

What should the assistant do?

A. Label it sterile if the cleaning bath was warm.
B. Store it as sterile because ultrasonic cleaning is the same as sterilization.
C. Skip packaging and release it based on visible cleanliness.
D. Continue the required post-cleaning reprocessing sequence before sterile release.

Q80 | Patient Intake and Vitals

SCENARIO A patient tells the rooming assistant, "I am thinking of hurting myself today." The clinic safety protocol requires staying with the patient and immediately summoning the clinician for this response.

What should the assistant do?

A. Finish the remaining routine screen before mentioning the statement.
B. Send the patient alone to the waiting area until an appointment slot opens.
C. Stay with the patient and immediately summon the clinician under the safety protocol.
D. Promise secrecy and leave the response out of the clinical record.

Q81 | General Patient Care

SCENARIO For a specific authorized adult intramuscular injection, the supervising provider's verified device table requires a 1-inch needle for this patient and site. Available needles have readable lengths of 0.5 inch, 0.625 inch, 1 inch, and 1.5 inches.

Which length follows this patient-specific instruction?

A. 0.625 inch
B. 0.5 inch
C. 1.5 inches
D. 1 inch

Q82 | Infection Control and Safety

SCENARIO For a designated room, the infection-prevention instruction requires an approved sporicidal product. Product A has the required compatible-use sporicidal claim; product B lacks that claim. Both have complete labels.

Which selection follows the instruction?

A. Use product A according to its full label instructions.
B. Use product B solely because it has a stronger fragrance.
C. Use either product without checking its labeled organism coverage.
D. Mix the two products to create the missing claim.

Q83 | Phlebotomy

SCENARIO For an authorized routine venipuncture, the verified device procedure specifies bevel-up orientation and an entry angle of 15-30 degrees. Before puncture, the assistant notices a bevel-down orientation at about 60 degrees.

Which correction matches the specified entry technique?

A. Turn the bevel up but retain the 60-degree angle.
B. Use the shallow angle but keep the bevel down.
C. Keep the bevel down and use the same 60-degree angle.
D. Reorient bevel up and use the specified 15-30-degree entry angle.

Q84 | Patient Intake and Vitals

> **SCENARIO** An intake form defines one 12 oz beer at 5% alcohol as one standard drink. A patient reports drinking two such beers on each of three evenings in a usual week.

How many standard drinks per usual week should be recorded from this report?

A. 6 standard drinks

B. 2 standard drinks

C. 3 standard drinks

D. 12 standard drinks

ANSWER KEY & EXPLANATIONS

Practice Exam 24 | Domain 3: Clinical Patient Care | 84 scored questions

01 C	02 D	03 D	04 A	05 D	06 B
07 C	08 A	09 D	10 A	11 D	12 A
13 C	14 A	15 B	16 D	17 A	18 B
19 D	20 C	21 A	22 B	23 A	24 D
25 B	26 C	27 C	28 B	29 A	30 B
31 A	32 D	33 B	34 B	35 D	36 B
37 C	38 C	39 B	40 B	41 A	42 C
43 B	44 B	45 B	46 A	47 C	48 D
49 C	50 B	51 C	52 D	53 A	54 B
55 B	56 C	57 D	58 B	59 B	60 D
61 B	62 B	63 C	64 A	65 A	66 D
67 C	68 D	69 A	70 C	71 C	72 C
73 A	74 B	75 A	76 D	77 C	78 A
79 D	80 C	81 D	82 A	83 D	84 A

DETAILED EXPLANATIONS

Q01 | Correct answer: C

Rotate and reapply the cuff so the marker aligns with the specified artery position.

The marker should follow the cuff's specified artery alignment. Increasing pressure or moving the stethoscope to an incorrect anatomical location does not correct cuff orientation.

Q02 | Correct answer: D

Secure the bag in its appropriate holder below bladder level without disconnecting the system.

The stated placement and closed-system requirements apply during transfer as well as at rest. A lap position, floor placement, or unneeded disconnection does not meet those conditions.

Q03 | Correct answer: D

Follow the approved dispenser procedure rather than topping off the partly filled soap.

Topping off liquid soap can introduce or support contamination. The approved dispenser replacement and cleaning process is preferable to adding soap or water to the residual contents.

Q04 | Correct answer: A

Treat it as contaminated and obtain a sterile replacement.

The supplied field boundary means the instrument contacted a nonsterile area. Moving it afterward or judging its appearance does not restore sterility.

Q05 | Correct answer: D

The validated compatible sterilization process through the approved service before use.

The device must meet both the sterility requirement and material compatibility instructions. An available but prohibited cycle or a lower processing level is not an equivalent substitute.

Q06 | Correct answer: B

Collect from the suitable right arm without independently changing the infusion.

The available opposite-arm site follows the stated specimen-collection procedure. This task does not authorize the assistant to manipulate an IV infusion or independently collect from its tubing.

Q07 | Correct answer: C

QC fails against the current 100-120 range; hold testing and follow corrective instructions.

The acceptance range must match the control lot actually used. Ninety is below the new lot's lower limit of 100 even though it would have passed a different lot's range.

Q08 | Correct answer: A

Remove the food from specimen storage and use the designated food-storage area.

Food should not be stored where blood or other potentially infectious materials are present. A lid or shelf separation does not make the specimen refrigerator an appropriate food-storage location.

Q09 | Correct answer: D

The 22 mm Hg systolic fall.

138 - 116 = 22 mm Hg, meeting the supplied systolic threshold; 82 - 76 = 6 mm Hg does not meet the diastolic threshold. The rule uses either pressure criterion and does not require symptoms.

Q10 | Correct answer: A

Pause administration and obtain clinician review of the new alert before proceeding.

A new safety alert requires reconciliation with the existing order before administration. An older signed order does not establish that the prescriber considered information entered afterward.

Q11 | Correct answer: D

Alternate five back blows with five abdominal thrusts as trained.

Current training for this described adult severe-obstruction situation uses alternating back blows and abdominal thrusts. If he becomes unresponsive, the response changes to trained CPR; blind finger sweeps should not be used.

Q12 | Correct answer: A

Establish the required operating conditions before testing under the assay instructions.

The environment does not meet the supplied operating range. Unopened packaging or specimen appearance does not authorize testing outside the method's stated conditions.

Q13 | Correct answer: C

11:35 a.m.

11:20 a.m. plus the specified 15-minute interval is 11:35 a.m. The baseline and post-medication sets should also retain their separate labels and times rather than be combined.

Q14 | Correct answer: A

Use the permitted temporal device and document the actual measurement route.

The oral technique cannot meet its stated condition, so the permitted alternative is appropriate. The result should retain its actual route rather than be relabeled or mathematically guessed.

Q15 | Correct answer: B

Signal when you first hear the tone, even if it is faint.

The response should reflect perception of the actual tone at the instructed time. Delayed or patterned responses do not provide the requested screening behavior.

Q16 | Correct answer: D

90 beats/min

9 beats x 60 seconds/min / 6 seconds = 90 beats/min. The full-window calculation estimates the mean over that interval; the raw count of 9 is not a one-minute rate.

Q17 | Correct answer: A

Squeeze as instructed to bend and lift the staple.

The device's specified bending action lifts the staple ends for removal. Simply pulling or rotating a closed staple does not use the remover as instructed and can injure tissue.

Q18 | Correct answer: B

Stop safely and use the approved lower-vacuum, small-volume approach under the collection plan.

The repeated collapse is linked to the applied collection vacuum rather than one defective tube. The supplied alternate-method plan addresses that mechanism without unnecessary probing or stronger suction.

Q19 | Correct answer: D

Left fifth intercostal space at the midclavicular line.

The usual adult cardiac apex is heard at the left fifth intercostal space near the midclavicular line. The upper sternal-border sites correspond to other cardiac listening areas rather than the usual apical pulse location.

Q20 | Correct answer: C

Reposition the patient or lighting so light falls onto the face rather than from behind it.

Lighting from behind can obscure the area the clinician needs to see. Adjusting the physical lighting or position improves the actual image rather than substituting unrelated connection changes.

Q21 | Correct answer: A

Use the cloth barrier and the prescribed 10-minute application period.

The provider's specified barrier and time limit govern this application. Direct prolonged exposure or an unapproved substitution changes the plan and can injure skin.

Q22 | Correct answer: B

Pass the closed scissors with the finger rings presented to the provider.

Presenting the rings with closed blades follows the specified controlled transfer. Directing open cutting tips toward the receiver creates an avoidable sharps hazard.

Q23 | Correct answer: A

Cleaning followed by the compatible low-level disinfection process.

A device contacting only intact skin is noncritical and follows the supplied low-level disinfection instructions. Cleaning alone omits disinfection, while an incompatible stronger process is not an appropriate substitute.

Q24 | Correct answer: D

Record product A with the left deltoid and product B with the right deltoid.

Each product should remain linked to its actual administration site. A shared nonspecific phrase or a lot number alone cannot distinguish which injection occurred in which arm.

Q25 | Correct answer: B

Record the patient's report and the measured temperature separately, and report the finding as required.

A subjective report and an objective measurement can differ and both should be preserved. Making them agree by deleting or changing information misrepresents what was reported and observed.

Q26 | Correct answer: C

Use the supplied collection container and follow the kit's preparation directions.

The collection method and materials must match the supplied kit instructions. Transferring afterward does not remove effects of an unapproved collection material.

Q27 | Correct answer: C

Document the patient's exact description and flag the unresolved dose for clinician verification.

Click increments are device-specific and cannot be converted without a reliable reference. Preserving the reported wording and identifying the uncertainty supports verification without inventing a dose.

Q28 | Correct answer: B

Retain the original accession and 9:00 a.m. collection time, and record 9:45 a.m. separately as aliquot preparation.

The aliquot comes from the original collection, so that identity and time remain relevant. Preparation time is additional information and does not reset the specimen's collection history.

Q29 | Correct answer: A

Eliminate the air pocket and fill the channel according to instructions, then complete the full exposure.

The air pocket prevents required contact with part of the channel. Completing the instructed filling step before the full exposure addresses that gap rather than guessing a time compensation.

Q30 | Correct answer: B

Place and secure the approved sharps container at the required point-of-use location.

Point-of-use disposal should be prepared before a contaminated sharp is generated. Planning an exposed carry or temporary unapproved container does not provide the required disposal setup.

Q31 | Correct answer: A

2 vials

The dose requires 600 mg / 300 mg/mL = 2 mL. One vial supplies only 1.5 mL, or 450 mg, so the minimum supply is two new vials; more vials are unnecessary for this dose.

Q32 | Correct answer: D

Release and inhale one puff at a time, following the prescribed interval and technique.

The supplied method uses a separate inhalation for each released puff. Combining puffs or independently increasing the dose does not follow the prescribed delivery technique.

Q33 | Correct answer: B

Preserve the originals and flag the verified clock offset for the reviewer.

A known clock offset is relevant to correlating events and should be made explicit. Preserving the original data allows an authorized reviewer to account for it without concealed changes.

Q34 | Correct answer: B

Obtain approved fitting guidance because neither available size matches both measurements.

The ankle matches medium, but the calf exceeds its range; large fits the calf but not the ankle. The supplied rule requires both measurements, so an authorized alternative fitting is needed rather than a one-measurement choice.

Q35 | Correct answer: D

Use the stated capillary sequence, with EDTA before serum.

The guide explicitly distinguishes its capillary sequence from venous collection. Following a different method's habit or transferring blood afterward does not meet the stated microcollection process.

Q36 | Correct answer: B

Choose the face that shows how much you hurt, even if your mood is different.

The supplied scale asks the child to report pain intensity rather than happiness or sadness. An observer-selected face or a default middle response replaces the child's actual self-report.

Q37 | Correct answer: C

Stop further attempts and contact the designated lead under the policy.

The attempt limit has been reached and applies despite use of fresh equipment. Escalating through the defined process avoids unauthorized repeated attempts or probing.

Q38 | Correct answer: C

Use the trained supportive hold and pause for help if a safe steady position cannot be maintained.

The described method supports safe positioning without restricting breathing. Uncontrolled limb movement or improvised force is not equivalent to the trained comfort hold.

Q39 | Correct answer: B

Obtain instructions for an acceptable specimen or appropriately authorized laboratory route without modifying the waived method.

The supplied specimen requirement is part of the test method. A different blood fraction cannot be treated as equivalent or made acceptable by relabeling or unvalidated modification.

Q40 | Correct answer: B

Flag the unmet requirement and obtain the procedural clinician's direction before sedation proceeds.

The stated prerequisite is not met and needs clinician resolution before sedation. The assistant should not invent an exception or treat access to a car as an appropriate escort.

Q41 | Correct answer: A

10:40 a.m.

8:40 a.m. plus 2 hours is 10:40 a.m. Collection timing should be based on the actual documented dose time, not the start of the clinic day.

Q42 | Correct answer: C

2.5 mL

5 g per 100 mL = 5,000 mg per 100 mL = 50 mg/mL. 125 mg / 50 mg/mL = 2.5 mL; treating the percentage as 5 mg/mL would give a tenfold larger volume.

Q43 | Correct answer: B

The specified chemical-resistant gloves.

The protection must match the chemical's stated compatibility requirement. Doubling an unsuitable material or shortening unprotected exposure does not establish the specified chemical barrier.

Q44 | Correct answer: B

Clean the stopper using the instructed aseptic preparation.

A protective vial cap is not a guarantee that the stopper needs no preparation. The supplied aseptic step still applies before needle entry.

Q45 | Correct answer: B

Allow the packages to cool fully in the designated area before handling them for storage.

The stated cooling step is part of safe post-cycle handling. Improvised wet cooling, tight stacking, or opening sterile barriers does not follow the validated handling process.

Q46 | Correct answer: A

The tuning fork.

A tuning fork provides a vibrating stimulus for the requested examination. A reflex hammer is intended to elicit reflexes and does not replace that stimulus.

Q47 | Correct answer: C

Enter the care now, retain the entry timestamp, and identify 10:00 a.m. as the actual service time.

A late entry should distinguish when care occurred from when documentation was entered. Preserving both times maintains an accurate sequence without altering the audit record.

Q48 | Correct answer: D

Use a new device and obtain a proper continuous fill under the instructions.

The visible bubble fails the supplied filling criterion. A fresh correctly filled device follows the method, whereas prohibited topping up or rough handling does not validate the sample.

Q49 | Correct answer: C

Check the recording connections and follow the unresolved-fault process if needed.

The patient findings do not match a cardiac-arrest conclusion based only on the display. After the immediate patient check, the stated recording-fault process is appropriate rather than treating an unverified device display as a diagnosis.

Q50 | Correct answer: B

Remove only the specified alternate sutures and leave the remainder for the ordered reassessment.

The order limits today's removal to a defined subset. A closed appearance does not authorize removal of the remaining sutures or a change in the follow-up plan.

Q51 | Correct answer: C

Relay the authenticated corrected result promptly and document it through the approved amendment workflow.

The clinician may still be acting on the superseded information, so the supplied active notification process matters. The assistant records the amendment without independently selecting a treatment change.

Q52 | Correct answer: D

10 electrodes

Four limb electrodes plus six precordial electrodes total ten physical electrodes. The number of derived leads is different from the count of electrodes attached to the patient.

Q53 | Correct answer: A

Prepare each product separately in its own labeled syringe.

Separately ordered products should remain separate when there is no licensed or instructed combination. The assistant must not create a combination vaccine by mixing products to reduce injections.

Q54 | Correct answer: B

The supplied nonalcohol skin preparation specified for the ethanol specimen.

The specified preparation is part of this assay's collection method. A routine alcohol pad should not be substituted or documented as a different product.

Q55 | Correct answer: B

Secure the remainder as directed and complete disposal with the required witness before signing the waste record.

The stated waste process requires actual witnessing and secure holding in the meantime. A later signature without observation or an assistant-signed witness field does not meet that requirement.

Q56 | Correct answer: C

Report an invalid monitoring run and follow the required review process.

The control did not demonstrate the viability required by the supplied instructions. The processed negative result therefore cannot be treated as a valid successful monitor result from this run.

Q57 | Correct answer: D

Prepare the solution within the specified temperature range before use.

The specified temperature is part of the product's effective use conditions. More heat or concentrate is not an instructed correction for a solution prepared outside that range.

Q58 | Correct answer: B

20/40

The patient was tested at 20 ft and read the 40 ft line, giving 20/40 under the stated convention. Reversing numerator and denominator changes the observation.

Q59 | Correct answer: B

Stop further oral administration and promptly obtain the required clinician review.

The new symptom triggers the stated review requirement before more oral medication is given. An existing oral order does not resolve a newly observed swallowing problem or authorize an unverified formulation change.

Q60 | Correct answer: D

Restart the anatomical count at the sternal angle and second rib as directed.

The supplied landmark method corrects the source of the counting error. An arbitrary shift or convenient surface landmark does not establish the intended intercostal positions.

Q61 | Correct answer: B

3.01 kg

10 oz / 16 = 0.625 lb; (6 + 0.625) lb / 2.2 lb/kg = 3.01136... kg, which rounds to 3.01 kg. Ignoring the ounces gives 2.73 kg and understates the measured weight.

Q62 | Correct answer: B

Use the listed compatible pediatric sensor and verify an adequate signal.

A correctly fitting compatible sensor follows the device's patient-size instructions. Padding or accepting an unstable display does not make an oversized sensor an appropriate fit.

Q63 | Correct answer: C

Keep the device out of patient acquisition and arrange the required equipment review.

The actual check output is below the stated 9 mm lower limit. A correct-looking setting does not override a failed functional verification, and improvised report adjustments are not the required correction.

Q64 | Correct answer: A

Apply the required product identity and hazard information before the bottle enters use or storage.

The supplied policy requires enough information to identify the product and its hazards. A person's name, color, or inaccurate product name does not provide that information.

Q65 | Correct answer: A

Clear the eyewash access path under the safety procedure.

The required emergency facility must be reachable when the hazardous task begins. Relying on low likelihood or an improvised later search does not provide the stated readiness.

Q66 | Correct answer: D

Shake the suspension as instructed before measuring the verified dose.

A suspension may require redistribution to provide the intended concentration in a measured dose. Selecting one separated layer or diluting the product changes the preparation rather than following its instructions.

Q67 | Correct answer: C

Seal the designated end with the supplied material and verify it before centrifugation.

The specified seal is a required containment step before this tube is centrifuged. A loose label or unsealed tube does not meet the supplied device preparation.

Q68 | Correct answer: D

Help the patient into the specified left-side position with the upper knee flexed.

The order supplies the intended left-lateral configuration for this examination. Prone, lithotomy, and upright sitting are different positions and do not meet it.

Q69 | Correct answer: A

Maintain the instructed clean-gauze pressure and confirm bleeding stops without adding a loose adhesive bandage.

The supplied procedure combines hemostasis with avoidance of a loose item the infant could place in the mouth. Omitting the bandage does not mean omitting pressure or the bleeding check.

Q70 | Correct answer: C

Obtain the sets from the two separately prepared ordered sites and document each actual site.

The order distinguishes independent site collections, not merely two labels or containers. Actual site documentation should correspond to how each set was collected.

Q71 | Correct answer: C

Clean outward from the incision with fresh gauze surfaces for successive passes.

The directed clean-to-outer sequence avoids bringing material back toward the incision with a used surface. The assistant should follow this specific wound-care order rather than improvise a different cleansing pattern.

Q72 | Correct answer: C

The designated combined biological-and-chemical hazard route.

Both hazards remain relevant to disposal under the supplied procedure. Treating the item only as ordinary biological waste or drain waste ignores the chemical component.

Q73 | Correct answer: A

Use the accessible scale with the specified chair-tare procedure.

An accessible scale permits weighing while following the patient's mobility needs and the device's tare method. An unsafe transfer or a reported estimate is not equivalent to this available measurement.

Q74 | Correct answer: B

Use the approved lens solution and lens paper.

Optical cleaning should use the specified nonabrasive materials. Abrasive towels, scraping, or an unapproved coating can damage or obscure the lens.

Q75 | Correct answer: A

Into the lower conjunctival sac as instructed.

The lower conjunctival sac is the specified delivery location. Touching the cornea with an applicator risks injury and contamination rather than improving delivery.

Q76 | Correct answer: D

Activate the clinic emergency response and summon immediate clinical help.

The observed color change meets the supplied emergency criterion. Routine variation in infant breathing does not justify delaying help for a pause with this concerning sign.

Q77 | Correct answer: C

158/82 mm Hg

The first sound is at 158 mm Hg and the final disappearance is at 82 mm Hg. The temporary silent interval does not replace either endpoint under the supplied procedure.

Q78 | Correct answer: A

Withhold oral intake and summon immediate clinical help under the emergency procedure.

The patient no longer meets the oral standing order's safety conditions. Immediate clinical escalation is needed; giving material by mouth to someone unable to swallow safely creates additional risk.

Q79 | Correct answer: D

Continue the required post-cleaning reprocessing sequence before sterile release.

Ultrasonic cleaning removes soil but does not itself complete the stated sterilization process. The remaining required steps must be completed before the instrument is released as sterile.

Q80 | Correct answer: C

Stay with the patient and immediately summon the clinician under the safety protocol.

The patient's statement triggers the supplied immediate safety response. Leaving the patient alone or delaying notification does not follow that protocol; the assistant should report the statement accurately.

Q81 | Correct answer: D

1 inch

The patient-specific table specifies a 1-inch needle. Another available length should not be substituted merely because it fits the syringe; the selection must follow the supplied verified instruction.

Q82 | Correct answer: A

Use product A according to its full label instructions.

The required claim is explicitly present on product A. A fragrance, an improvised mixture, or a general disinfectant label without the needed claim does not meet the stated requirement.

Q83 | Correct answer: D

Reorient bevel up and use the specified 15-30-degree entry angle.

Both the bevel orientation and the angle must match the supplied device procedure. Correcting only one of the two observed errors leaves the other technique requirement unmet.

Q84 | Correct answer: A

6 standard drinks

2 drinks per evening x 3 evenings per week = 6 standard drinks per week under the supplied definition. This records the reported quantity without assigning a diagnosis.

End of Practice Exam